NCE EXAM PREP

Practice Exam 20

Domain 2

Intake & Assessment

30 Practice Questions | 40 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 04. Use the suggested 40-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

When the confidence level is increased while estimating the range around an assessment score, what generally happens to the confidence interval if other factors remain constant?

A) It becomes narrower

B) It becomes wider

C) It becomes identical to the observed score

Question 2

> A clinic wants a brief measure to track weekly change in depressive symptoms among adults receiving outpatient counseling. Several candidate measures are available.

Which qualities are most important for this purpose?

A) A long administration time and items covering every psychiatric diagnosis

B) Reliability, validity for the intended population and use, and sensitivity to clinically meaningful change

C) Popularity with clinicians even when evidence about change over time is unavailable

Question 3

> A client referred by a faith leader says spiritual beliefs are important and may be relevant to counseling. The client is unsure whether to invite the faith leader into any sessions and does not want the counselor to assume a religious counseling approach.

What is the most appropriate intake response?

A) Contact the faith leader immediately because the referral implies permission

B) Ask how the client wants spirituality addressed, clarify the counselor's role, and discuss consent before any outside involvement

C) Exclude all spiritual content because it falls outside counseling assessment

Question 4

A counselor is authorized to provide telehealth only when a client is physically located in State A. A prospective client lives in State A but frequently works from State B and asks to attend video sessions from either location. The applicable practice rule is stated clearly in the clinic policy.

What should the counselor establish during intake?

A) The client's location at each session, scheduling within the authorized jurisdiction, and referral or alternative plans for times the client is in State B

B) Only the client's permanent home address because physical location during a session is irrelevant

C) A promise that the counselor's State A authorization automatically extends wherever the client travels

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Question 5

An observational measure has high interrater reliability but poor evidence of validity. What is the best conclusion?

A) The measure must be valid because different raters agree

B) Raters score consistently, but the measure may not assess the intended construct accurately

C) The measure is invalid only when the same rater changes scores over time

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Question 6

A client with no prior anxiety history develops restlessness, insomnia, weight loss despite increased appetite, heat intolerance, tremor, and a rapid heartbeat. The symptoms have intensified over six weeks, and the client has not seen a medical provider.

What is the best assessment decision?

A) Diagnose generalized anxiety disorder because restlessness and insomnia are present

B) Begin exposure-based treatment before asking about physical health

C) Arrange medical evaluation for a possible physiological contributor while continuing the mental health assessment

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Question 7

Two months after a violent home invasion, a client reports recurrent involuntary memories, nightmares, avoidance of the neighborhood, persistent guilt, emotional detachment, exaggerated startle, sleep disturbance, and substantial occupational impairment. The symptoms began immediately after the event and are not substance induced.

Which diagnosis is best supported by the information?

A) Acute stress disorder

B) Adjustment disorder with anxiety

C) Posttraumatic stress disorder

Question 8

A client joins a video intake from a parked car at the workplace. Coworkers periodically walk past, and the client says a supervisor may open the car door using a spare key. The client plans to discuss a sensitive workplace conflict.

What should the counselor do before continuing the sensitive discussion?

A) Review the privacy risks and options with the client and determine whether to relocate, reschedule, or proceed with an agreed privacy plan

B) Continue because a parked car is automatically considered a private clinical setting

C) End all telehealth services permanently because the first location is imperfect

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Question 9

A client reports a suicide attempt three days ago and now describes a specific plan to make another attempt tonight. The client has immediate access to the identified means, cannot identify a reason to delay, and refuses voluntary safety arrangements.

What is the most appropriate response?

A) Schedule a routine follow-up in one week because the client disclosed the plan voluntarily

B) Initiate emergency intervention and arrange evaluation at a level of care capable of maintaining immediate safety

C) Rely on a written promise that the client will call if the plan becomes more serious

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Question 10

In diagnostic screening, what does specificity describe?

A) The proportion of people with the condition who receive a positive result

B) The stability of scores when the same people are retested

C) The proportion of people without the condition who receive a negative result

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Question 11

Three adult siblings attend a family intake. One says participation resulted from family pressure, does not agree to counseling, and will not discuss personal information. The other siblings ask the counselor to proceed and require that sibling to remain.

What should the counselor do?

A) Clarify voluntary participation, privacy and information expectations, and possible service formats before deciding whether conjoint work can proceed

B) Begin family counseling because a majority of the siblings consented

C) Tell the reluctant sibling that attendance creates automatic consent to disclose personal information

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Question 12

A client begins counseling in an integrated primary care clinic and assumes that counseling documentation is hidden from every other member of the care team. Clinic policy permits specified team members to access relevant portions of the shared health record for coordinated care.

What should the counselor do during intake?

A) Confirm that no team member can ever access counseling information

B) Avoid discussing the shared record unless another provider later opens it

C) Explain the record arrangement, who may access relevant information, the purposes and limits of sharing, and any available client choices

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Question 13

During a mental status examination, the counselor asks the client to state the client's name, current location, date, and reason for being evaluated. The client answers all four accurately.

Which finding is best documented?

A) Intact remote memory

B) Adequate abstract reasoning

C) Orientation to person, place, time, and situation

Question 14

A client wants to use health insurance for counseling but believes the insurer will receive no information at all. The plan requires the clinic to submit the service date and a diagnosis with each claim, and the clinic also offers a self-pay option.

What should the counselor explain before the client chooses a payment method?

A) The information required for insurance claims, relevant confidentiality limits, and the available self-pay option

B) Insurance billing never involves disclosure of diagnostic information

C) The client must use insurance because payment preferences are unrelated to informed consent

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Question 15

A new client arrives for intake visibly intoxicated, cannot follow a basic explanation of confidentiality, and repeatedly falls asleep. A sober friend is available to drive the client home, and no acute medical emergency is apparent.

What should the counselor do?

A) Address immediate safety and transportation, defer full informed consent and routine intake until the client can participate meaningfully, and provide appropriate urgent-resource information

B) Obtain a signature while the client is awake briefly and complete the entire intake

C) Ask the friend to consent to counseling on the client's behalf without determining any legal authority

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Question 16

> A personality inventory produces validity indicators suggesting highly inconsistent responding. The client says severe fatigue and difficulty understanding several items made it hard to answer carefully.

What is the most appropriate interpretation?

A) Interpret every clinical scale as accurate because validity indicators do not affect score meaning

B) Conclude that the client intentionally fabricated symptoms because inconsistency has only one possible cause

C) Investigate the administration conditions and response validity before drawing conclusions from the clinical scales

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Question 17

> Six weeks after an unexpected job loss, a client reports tearfulness, worry, poor sleep, and difficulty concentrating. The response causes impairment but does not meet criteria for another disorder, and there was no similar syndrome before the job loss.

Which diagnosis is most consistent with the information?

A) Persistent depressive disorder

B) Adjustment disorder with mixed anxiety and depressed mood

C) Generalized anxiety disorder

Question 18

> During intake, a counselor realizes that the prospective client is the spouse of a close friend whom the counselor sees socially every week. The relationship could expose private information and impair the counselor's objectivity, and other qualified counselors are available.

What is the best intake action?

A) Accept the client without discussion because the counselor has not previously met the spouse

B) Explain the relevant conflict and privacy concerns without disclosing unnecessary information and facilitate an appropriate referral

C) Ask the close friend for permission to provide counseling to the spouse

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Question 19

> A client reviews an online intake summary and notices that an old address, an outdated emergency contact, and an incorrect relationship status were imported automatically. The counselor has not yet relied on the summary for assessment.

What should the counselor do?

A) Leave the entries unchanged because electronically imported information is more reliable than client correction

B) Verify the current information with the client, correct the record through the appropriate process, and clarify how the information will be used

C) Delete the entire record and require the client to repeat every intake form regardless of relevance

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Question 20

> For four months, a client has had weekly episodes of eating unusually large amounts of food with a sense of loss of control, rapid eating, eating when not hungry, and intense shame afterward. The client reports marked distress and no regular compensatory behaviors.

Which diagnosis is best supported by the information?

A) Binge-eating disorder

B) Bulimia nervosa

C) Anorexia nervosa, binge-eating/purging type

Question 21

> For nine months, a client has feared using public transportation, standing in crowded lines, being in enclosed theaters, and leaving home alone because escape or help might be difficult if panic-like symptoms occur. The fear has led to extensive avoidance and significant impairment.

Which diagnosis is best supported by the information?

A) Social anxiety disorder

B) Specific phobia, situational type

C) Agoraphobia

Question 22

> A client reports new episodes of jitteriness, palpitations, insomnia, and gastrointestinal discomfort. A biopsychosocial interview reveals daily consumption of eight high-caffeine energy drinks, with symptoms peaking soon after use and improving when intake decreases.

What is the best diagnostic assessment approach?

A) Examine the temporal relationship to caffeine and assess for a substance-induced anxiety presentation before assigning a primary anxiety disorder

B) Diagnose panic disorder because palpitations and jitteriness are sufficient criteria

C) Ignore caffeine because common legal substances cannot cause clinically significant anxiety symptoms

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Question 23

> A client reports increasingly frequent online sports betting, repeated unsuccessful efforts to stop, lying to family about losses, borrowing money, and returning the day after a loss to try to win it back. The client appears ashamed and says debt has become overwhelming.

What is the most appropriate assessment response?

A) Treat the behavior only as a budgeting problem because no substance is involved

B) Assess gambling-disorder features, financial and relationship consequences, mood, and suicide risk

C) Diagnose a manic episode solely because the client has spent large amounts of money

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Question 24

> A client developed delusions, auditory hallucinations, and disorganized speech after an extreme nontraumatic stressor. Symptoms lasted 18 days, were followed by full return to prior functioning, and were not attributable to substances, a medical condition, or a mood disorder.

Which diagnosis best fits the course described?

A) Brief psychotic disorder

B) Schizophreniform disorder

C) Schizophrenia

Question 25

> A referral form labels a prospective client with borderline personality disorder. During intake, the client disputes the label and says the referring program made it after a single brief meeting. No supporting assessment records have been provided.

What is the counselor's best response?

A) Obtain the client's account, conduct an independent assessment, and seek relevant records with permission rather than adopting or rejecting the label automatically

B) Copy the diagnosis into the record as confirmed because it appears on a referral form

C) Promise the client that the diagnosis is wrong before gathering any additional information

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Question 26

Which measure of central tendency is generally least affected by a few extremely high or low scores?

A) Mean

B) Mode

C) Median

Question 27

> A transgender client gives a chosen name and pronouns that differ from the legal name required on insurance claims. The electronic record can display the chosen name prominently while retaining the legal name in designated billing fields.

What is the counselor's best intake action?

A) Use only the legal name in every interaction because the insurer requires it on claims

B) Confirm and document the client's name and pronouns, use them in care, and explain where the legal name must appear for billing

C) Avoid recording any name until the insurer decides which one the counselor should use in sessions

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Question 28

On a scale with a T-score mean of 50 and a standard deviation of 10, what does a T-score of 70 represent?

A) Two standard deviations above the mean

B) Two standard deviations below the mean

C) The 70th percentile in every norm group

Question 29

> For three years, a client has experienced depressed mood more days than not, low energy, low self-esteem, poor concentration, and hopelessness. The client has not been symptom free for longer than two months at a time, and substances, medical causes, and bipolar disorders have been ruled out.

Which diagnosis is most consistent with this pattern?

A) Major depressive disorder with seasonal pattern

B) Cyclothymic disorder

C) Persistent depressive disorder

Question 30

In a screening program, which result is a false positive?

A) A person with the condition receives a positive screen

B) A person with the condition receives a negative screen

C) A person without the condition receives a positive screen

PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: B

Correct Answer: It becomes wider

Explanation: A higher confidence level requires a broader interval to increase the likelihood that the interval contains the underlying score. The interval therefore generally widens when other factors remain constant.

Question 2 — Correct: B

Correct Answer: Reliability, validity for the intended population and use, and sensitivity to clinically meaningful change

Explanation: A progress-monitoring measure must produce dependable and meaningful scores and be responsive to change over the selected interval. Length or popularity cannot substitute for evidence supporting the intended use.

Question 3 — Correct: B

Correct Answer: Ask how the client wants spirituality addressed, clarify the counselor's role, and discuss consent before any outside involvement

Explanation: Spirituality can be explored according to the client's preferences without assuming a particular counseling approach. Referral by a faith leader does not authorize contact or participation without the client's consent.

Question 4 — Correct: A

Correct Answer: The client's location at each session, scheduling within the authorized jurisdiction, and referral or alternative plans for times the client is in State B

Explanation: The stated rule turns physical location during each session into a condition of service. Intake should establish verification and contingency procedures rather than relying only on residence or assuming authorization follows the client.

Question 5 — Correct: B

Correct Answer: Raters score consistently, but the measure may not assess the intended construct accurately

Explanation: Reliability and validity answer different questions. Agreement among raters supports consistency, but it does not establish that the resulting scores represent the construct the measure claims to assess.

Question 6 — Correct: C

Correct Answer: Arrange medical evaluation for a possible physiological contributor while continuing the mental health assessment

Explanation: Weight loss with increased appetite, heat intolerance, tremor, and tachycardia raises concern for a physiological cause of anxiety-like symptoms. A primary anxiety diagnosis or treatment plan should not precede appropriate medical evaluation under these facts.

Question 7 — Correct: C

Correct Answer: Posttraumatic stress disorder

Explanation: The client has intrusion, avoidance, negative cognition or mood, and arousal symptoms with impairment persisting for more than one month after trauma. Acute stress disorder is limited to the earlier posttrauma period, while the symptom pattern is more specific than adjustment disorder.

Question 8 — Correct: A

Correct Answer: Review the privacy risks and options with the client and determine whether to relocate, reschedule, or proceed with an agreed privacy plan

Explanation: The location presents concrete risks to confidential discussion, but those risks can be reviewed collaboratively. Automatically proceeding or permanently ending telehealth would bypass available privacy and scheduling options.

Question 9 — Correct: B

Correct Answer: Initiate emergency intervention and arrange evaluation at a level of care capable of maintaining immediate safety

Explanation: A recent attempt, current specific plan, immediate means access, same-night timing, and refusal of safety steps indicate acute suicide risk. Routine follow-up or a written promise cannot provide the necessary level of protection.

Question 10 — Correct: C

Correct Answer: The proportion of people without the condition who receive a negative result

Explanation: Specificity is the true-negative rate among people who do not have the condition. The proportion of affected people who screen positive is sensitivity, and retest stability is a form of reliability.

Question 11 — Correct: A

Correct Answer: Clarify voluntary participation, privacy and information expectations, and possible service formats before deciding whether conjoint work can proceed

Explanation: Physical attendance does not create informed consent, especially when an adult explicitly refuses participation. The counselor should clarify roles and options before determining whether any conjoint or alternative format is suitable.

Question 12 — Correct: C

Correct Answer: Explain the record arrangement, who may access relevant information, the purposes and limits of sharing, and any available client choices

Explanation: The shared-record structure materially affects privacy expectations and coordinated care. It should be explained before services rather than concealed or described as completely inaccessible to the team.

Question 13 — Correct: C

Correct Answer: Orientation to person, place, time, and situation

Explanation: The four questions directly sample awareness of identity, location, date, and current circumstances. They do not primarily test recall of distant events or abstract reasoning.

Question 14 — Correct: A

Correct Answer: The information required for insurance claims, relevant confidentiality limits, and the available self-pay option

Explanation: The client needs accurate information about required claim disclosures before choosing to use insurance. Explaining the available payment options supports an informed decision without falsely promising complete nondisclosure.

Question 15 — Correct: A

Correct Answer: Address immediate safety and transportation, defer full informed consent and routine intake until the client can participate meaningfully, and provide appropriate urgent-resource information

Explanation: The client is not currently able to understand and participate in informed consent. Safety and transportation can be addressed now, but a signature or a friend's unauthorized substitute consent would not make the routine intake valid.

Question 16 — Correct: C

Correct Answer: Investigate the administration conditions and response validity before drawing conclusions from the clinical scales

Explanation: Inconsistent responding can make clinical-scale interpretation unsound, and fatigue or comprehension problems are plausible contributors. The indicators do not prove deliberate fabrication, so administration conditions require clarification before conclusions are drawn.

Question 17 — Correct: B

Correct Answer: Adjustment disorder with mixed anxiety and depressed mood

Explanation: Emotional and behavioral symptoms began soon after an identifiable stressor, cause impairment, and do not meet criteria for another disorder. The short duration and mixed response do not support persistent depressive disorder or generalized anxiety disorder.

Question 18 — Correct: B

Correct Answer: Explain the relevant conflict and privacy concerns without disclosing unnecessary information and facilitate an appropriate referral

Explanation: The close ongoing relationship creates foreseeable objectivity and privacy problems at the outset, and alternatives are available. Seeking the friend's permission would disclose the spouse's request and would not resolve the counselor's conflict.

Question 19 — Correct: B

Correct Answer: Verify the current information with the client, correct the record through the appropriate process, and clarify how the information will be used

Explanation: Imported data can be outdated or wrong and should be verified before being incorporated into assessment decisions. Targeted correction preserves record accuracy without treating electronic data as infallible or discarding all relevant intake work.

Question 20 — Correct: A

Correct Answer: Binge-eating disorder

Explanation: Recurrent weekly binge episodes with loss of control, associated features, marked distress, and at least three months' duration support binge-eating disorder. The absence of regular compensatory behavior distinguishes it from bulimia nervosa.

Question 21 — Correct: C

Correct Answer: Agoraphobia

Explanation: The client fears and avoids multiple situations because escape or help might be difficult if symptoms occur, and the pattern has persisted with impairment. The breadth of situations and escape-related concern exceed a single situational phobia and are not centered on social evaluation.

Question 22 — Correct: A

Correct Answer: Examine the temporal relationship to caffeine and assess for a substance-induced anxiety presentation before assigning a primary anxiety disorder

Explanation: Heavy caffeine exposure and the close symptom timing make a substance-related explanation central to the differential. Legal and commonly used substances can still produce clinically significant anxiety symptoms.

Question 23 — Correct: B

Correct Answer: Assess gambling-disorder features, financial and relationship consequences, mood, and suicide risk

Explanation: Chasing losses, concealment, unsuccessful efforts to stop, borrowing, and serious debt support focused assessment for gambling disorder. Financial crisis and shame also make mood and suicide-risk assessment clinically relevant rather than reducing the problem to budgeting.

Question 24 — Correct: A

Correct Answer: Brief psychotic disorder

Explanation: The psychotic syndrome lasted more than one day but less than one month and was followed by full return to prior functioning. Schizophreniform disorder requires a longer duration, and schizophrenia requires an even more extended course.

Question 25 — Correct: A

Correct Answer: Obtain the client's account, conduct an independent assessment, and seek relevant records with permission rather than adopting or rejecting the label automatically

Explanation: A referral diagnosis is one data point rather than a confirmed conclusion. Independent assessment and appropriately authorized records allow the counselor to evaluate both the client's concern and the earlier formulation.

Question 26 — Correct: C

Correct Answer: Median

Explanation: The median depends on the ordered middle position and is relatively resistant to extreme scores. The mean can be pulled substantially by outliers.

Question 27 — Correct: B

Correct Answer: Confirm and document the client's name and pronouns, use them in care, and explain where the legal name must appear for billing

Explanation: The record can respect the client's identity while meeting the stated billing requirement. Transparent explanation prevents unexpected legal-name use without allowing an administrative field to dictate how the counselor addresses the client.

Question 28 — Correct: A

Correct Answer: Two standard deviations above the mean

Explanation: A T-score of 70 is 20 points above the mean of 50. With a standard deviation of 10, that difference equals two standard deviations and does not by itself specify a universal percentile.

Question 29 — Correct: C

Correct Answer: Persistent depressive disorder

Explanation: Depressed mood with associated symptoms has persisted for three years without a symptom-free interval longer than two months. The chronic course and stated exclusions support persistent depressive disorder rather than a seasonal or bipolar-spectrum pattern.

Question 30 — Correct: C

Correct Answer: A person without the condition receives a positive screen

Explanation: A false positive occurs when the test indicates the condition in someone who does not have it. A negative result in an affected person is a false negative.
